# Residential Treatment Facility
# Inspection Report

Department of Health
P.O. Box 47874, Olympia, WA 98504-7874

January 30, 2025

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| Two Rivers Landing, 504 S 3rd Ave, Yakima, WA. 98902-3543 | Edie Dibble |
| Agency Name and Address | Administrator |

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| ONGOING - ROUTINE | 10/14/24 | GLD03/ DEC13 |
| Inspection Type | Inspection Onsite Dates | Inspector |
| X2024-799 | RTF.FS.60068796 | Mental Health Services, |
| Inspection Number | License Number | RTF Service Types |

Please note that the deficiencies/violations/observations noted in this report are not all-inclusive, but rather were deficiencies/violations/observations that were observed or discovered during the on-site inspection.

| Deficiency Number and Rule Reference | Observation Findings | Plan of Correction |
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| 0630 Management of human resources<br>WAC 246-337-050(5)(c)(i) (5) The licensee must document that staff receive the following training as applicable: (c) If restraint or seclusion is used in the facility, initial and annual training in the proper and safe use of restraint or seclusion for staff required to perform restraint or seclusion procedures inclusive of: (i) Techniques to identify staff and resident behaviors, events, and environmental factors that may trigger circumstances that require the use of restraint or seclusion. | Based on personnel record review, and interview, it was determined the agency failed to ensure staff completed initial training that included proper and safe use of restraint or seclusion including techniques to identify staff and resident behaviors, events, and environmental factors that may trigger circumstances that require the use of restraint or seclusion.<br><br>Failure to ensure staff are competent by training, experience and capability places the client at risk significant of harm.<br><br>Findings included: | **How the Deficiency Will Be Corrected:**<br>• All Residential and Inpatient staff receive an initial ProACT training as well as annual boosters.<br>• The ProACT training covers all elements of WAC 246-337-050(5)(c).<br>• HR will ensure that the initial ProACT training is scheduled on date of hire, to be completed within 90-days of hire. |

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| | 1. Review of P&Ps submitted failed to address orientation training and staff training plans.<br><br>2. Review of the personnel record for Staff A, RN, date of hire 06/05/23, showed no techniques to identify staff and resident behaviors, events, and environmental factors that may trigger circumstances that require the use of restraint or seclusion training completed until ProACT training on 10/11/24, 16 months after hire.<br><br>3. Review of the personnel record for Staff, B, BHA, date of hire 07/10/23, showed no techniques to identify staff and resident behaviors, events, and environmental factors that may trigger circumstances that require the use of restraint or seclusion until ProACT training on 12/20/23, five months after hire.<br><br>4. During an interview on 10/14/24 at 10:26 AM with Staff, H, HR, they indicated they do not know why the training would not have been completed. They did indicate the staff may have had to wait until the agency had a trainer scheduled to provide the training. | • New evening ProACT training times have been established to accommodate night shift staff.<br>• ProACT training is now offered more frequently.<br><br>**Who is responsible:**<br>• Senior Vice President for Human Resources<br>• VP for Residential and Inpatient Services and acting Program Manager for Two Rivers Landing<br><br>**Date of Correction:**<br>• Completed as of 2/10/25.<br><br>**Monitors put in place to assure continuing compliance:**<br>• Relias (our online training system) will automatically send supervisors pending and overdue training reminders on a weekly basis. |
| 0635 Management of human resources<br>WAC 246-337-050(5)(c)(ii) (5) The licensee must document that staff receive the following training as applicable: (c) If restraint or seclusion is used in the facility, initial and annual training in the proper and safe use of restraint or seclusion for staff required to perform restraint or seclusion procedures inclusive of: (ii) The use of nonphysical intervention skills. | Based on personnel record review, and interview, it was determined the agency failed to ensure staff completed initial training that include proper and safe use of restraint or seclusion, including the use of nonphysical intervention skills.<br><br>Failure to ensure staff are competent by training, experience and capability places the client at risk significant of harm.<br><br>Findings included: | **See correction plan above.** |

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| | 1. Review of P&Ps submitted failed to address orientation training and staff training plans.<br><br>2. Review of the personnel record for Staff A, RN, date of hire 06/05/23, showed no use of nonphysical intervention skills training completed until ProACT training on 10/11/24, 16 months after hire.<br><br>3. Review of the personnel record for Staff B, BHA, date of hire 07/10/23, showed no use of nonphysical intervention skills until ProACT training on 12/20/23, five months after hire.<br><br>4. During an interview on 10/14/24 at 10:26 AM with Staff H, HR, they indicated they do not know why the training would not have been completed. They did indicate the staff may have had to wait until the agency had a trainer scheduled to provide the training. | |
| 0640 Management of human resources<br>WAC 246-337-050(5)(c)(iii) (5) The licensee must document that staff receive the following training as applicable: (c) If restraint or seclusion is used in the facility, initial and annual training in the proper and safe use of restraint or seclusion for staff required to perform restraint or seclusion procedures inclusive of: (iii) Choosing the least restrictive intervention based on an individualized assessment of the resident's medical or behavioral status or condition. | Based on personnel record review, and interview, it was determined the agency failed to ensure staff completed initial training that included proper and safe use of restraint or seclusion, including choosing the least restrictive intervention based on an individualized assessment of the resident's medical or behavioral status or condition.<br><br>Failure to ensure staff are competent by training, experience and capability places the client at risk significant of harm.<br><br>Findings included:<br><br>1. Review of P&Ps submitted failed to address orientation training and staff training plans. | **See correction plan above.** |

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| | 2. Review of the personnel record for Staff A, RN, date of hire 06/05/23, showed no training that included, choosing the least restrictive intervention based on an individualized assessment of the resident's medical or behavioral status or condition until ProACT training on 10/11/24, 16 months after hire.<br><br>3. Review of the personnel record for Staff B, BHA, date of hire 07/10/23, showed no training that included choosing the least restrictive intervention based on an individualized assessment of the resident's medical or behavioral status or condition until ProACT training on 12/20/23, five months after hire.<br><br>4. During an interview on 10/14/24 at 10:26 AM with Staff H, HR, they indicated they do not know why the training would not have been completed. They did indicate the staff may have had to wait until the agency had a trainer scheduled to provide the training. | |
| 0645 Management of human resources<br>WAC 246-337-050(5)(c)(iv) (5) The licensee must document that staff receive the following training as applicable: (c) If restraint or seclusion is used in the facility, initial and annual training in the proper and safe use of restraint or seclusion for staff required to perform restraint or seclusion procedures inclusive of: (iv) The safe application and use of all types of restraint or seclusion used in the RTF, including training in how to recognize and respond to signs of physical and psychological distress. | Based on personnel record review, and interview, it was determined the agency failed to ensure staff complete initial training that included the safe application and use of all types of restraint or seclusion used in the RTF, including training in how to recognize and respond to signs of physical and psychological distress.<br><br>Failure to ensure staff are competent by training, experience and capability places the client at risk significant of harm.<br><br>Findings included:<br><br>1. Review of P&Ps submitted failed to address orientation training and staff training plans. | **See correction plan above.** |

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| | 2. Review of the personnel record for Staff A, RN, date of hire 06/05/23, showed no training that included the safe application and use of all types of restraint or seclusion used in the RTF, including training in how to recognize and respond to signs of physical and psychological distress until ProACT training on 10/11/24, 16 months after hire.<br><br>3. Review of the personnel record for Staff B, BHA, date of hire 07/10/23, showed no training that included the safe application and use of all types of restraint or seclusion used in the RTF, including training in how to recognize and respond to signs of physical and psychological distress until ProACT training on 12/20/23, five months after hire.<br><br>4. During an interview on 10/14/24 at 10:26 AM with Staff H, HR, they indicated they do not know why the training would not have been completed. They did indicate the staff may have had to wait until the agency had a trainer scheduled to provide the training. | |
| 0650 Management of human resources<br>WAC 246-337-050(5)(c)(v) (5) The licensee must document that staff receive the following training as applicable: (c) If restraint or seclusion is used in the facility, initial and annual training in the proper and safe use of restraint or seclusion for staff required to perform restraint or seclusion procedures inclusive of: (v) Clinical identification of specific behavioral changes that indicate that restraint or seclusion is no longer necessary. | Based on personnel record review, and interview, it was determined the agency failed to ensure staff completed initial training that included clinical identification of specific behavioral changes that indicate that restraint or seclusion is no longer necessary.<br><br>Failure to ensure staff are competent by training, experience and capability places the client at risk significant of harm.<br><br>Findings included:<br>1. Review of P&Ps submitted failed to address orientation training and staff training plans. | **See correction plan above.** |

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| | 2. Review of the personnel record for Staff A, RN, date of hire 06/05/23, showed no training that included clinical identification of specific behavioral changes that indicate that restraint or seclusion is no longer necessary until ProACT training on 10/11/24, 16 months after hire.<br><br>3. Review of the personnel record for Staff B, BHA, date of hire 07/10/23, showed no training that included clinical identification of specific behavioral changes that indicate that restraint or seclusion is no longer necessary until ProACT training on 12/20/23, five months after hire.<br><br>4. During an interview on 10/14/24 at 10:26 AM with Staff H, HR, they indicated they do not know why the training would not have been completed. They did indicate the staff may have had to wait until the agency had a trainer scheduled to provide the training. | |
| 0655 Management of human resources<br>WAC 246-337-050(5)(c)(vi) (5) The licensee must document that staff receive the following training as applicable: (c) If restraint or seclusion is used in the facility, initial and annual training in the proper and safe use of restraint or seclusion for staff required to perform restraint or seclusion procedures inclusive of: (vi) Monitoring the physical and psychological well-being of the resident who is restrained or secluded including, but not limited to, respiratory and circulatory status, skin integrity, and vital signs. | Based on personnel record review, and interview, it was determined the agency failed to ensure staff completed initial training that included monitoring the physical and psychological well-being of the resident who is restrained or secluded including, but not limited to, respiratory and circulatory status, skin integrity, and vital signs.<br><br>Failure to ensure staff are competent by training, experience and capability places the client at risk significant of harm.<br><br>Findings included:<br><br>1. Review of P&Ps submitted failed to address orientation training and staff training plans. | **See correction plan above.** |

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| | 2. Review of the personnel record for Staff A, RN, date of hire 06/05/23, showed no training that included monitoring the physical and psychological well-being of the resident who is restrained or secluded including, but not limited to, respiratory and circulatory status, skin integrity, and vital signs until ProACT training on 10/11/24, 16 months after hire.<br><br>3. Review of the personnel record for Staff B, BHA, date of hire 07/10/23, showed no training that included monitoring the physical and psychological well-being of the resident who is restrained or secluded including, but not limited to, respiratory and circulatory status, skin integrity, and vital signs until ProACT training on 12/20/23, five months after hire.<br><br>4. During an interview on 10/14/24 at 10:26 AM with Staff H, HR, they indicated they do not know why the training would not have been completed. They did indicate the staff may have had to wait until the agency had a trainer scheduled to provide the training. | |
| 0800 Management of human resources<br>WAC 246-337-050(7)(h) (7) The licensee shall have written documentation for each staff member including: (h) Current Washington state food and beverage service worker permit, if applicable. | Based on personnel record review and interviews, it was determined that the agency failed to ensure all staff required by position description to have a current Washington state food and beverage permit was kept current.<br><br>Failure to ensure staff permits are current places clients at risk of harm due to staff providing food and beverage service they are not credentialed to provide.<br><br>Findings included: | **How the Deficiency Will Be Corrected:**<br>Our policy on "Food Preparation in Comprehensive Healthcare Residential Facilities" (PC-058) will be updated to indicate that a food and beverage permit will be up to date and posted for all staff involved in food preparation.<br><br>During onboarding, food and beverage permits will be collected from the employee. If an employee |

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| | 1. No P&P was submitted for review at the time of this writing.<br><br>2. Review of personnel records for Staff C, BHA showed a food handlers card that expired as of 06/17/24.<br><br>3. During an interview on 10/14/24 at 10:26 AM with Staff H, HR, they stated they were unable to find a current card for the staff member and there is a system in place to remind and receive the current card. | does not have a current permit, the employee will have 7 days to complete the training and present the current permit.<br><br>**Who is responsible:**<br>• Senior Vice President for Human Resources<br>• VP for Residential and Inpatient services and acting Program Manager for Two Rivers Landing<br><br>**Date of Correction:**<br>• Policy updated: By 2/28/2025<br>• Relias reminder changed from 30 days to 7 days: By 2/14/2025<br><br>**Monitors put in place to assure continuing compliance:**<br>• All current staff records will be reviewed to confirm food and beverage permit is in personnel file and current.<br>• Copies of the current permits will be posted in the kitchen.<br>• Relias (our online training system) will automatically send supervisors pending and overdue permits notification on a weekly basis. |
| 0805 Management of human resources<br>WAC 246-337-050(7)(i) (7) The licensee shall have written documentation for each staff | Based on personnel record review and interviews, it was determined that the agency failed to ensure all staff required by position description to have a current Washington driver's license was kept current. | **How the Deficiency Will Be Corrected:**<br>A copy of our current Vehicle Use Policy (HR-023) that covers when |

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| member including: (i) Current driver's license, if applicable. | Failure to ensure staff licenses are current places clients at risk of harm due to staff providing transportation services with no valid driver's license.<br><br>Findings included:<br><br>1. No P&P was submitted for review at the time of this writing.<br><br>2. Review of personnel records for Staff B, BHA showed a food handlers card that expired as of 06/17/24.<br><br>3. During an interview on 10/14/24 at 10:26 AM with Staff H, HR, they stated they were unable to find a current card for the staff member and there is a system in place to remind and receive the current card. | employees must submit a driver's license is included with this submission.<br><br>The submission of updated driver's licenses is tracked in Relias (our online reporting system).<br><br>**Who is responsible:**<br>• Senior Vice President for Human Resources<br><br>**Date of Correction:**<br>Completed as of 2/10/25.<br><br>**Monitors put in place to assure continuing compliance:**<br>• Relias (our online training system) will automatically send supervisors pending and overdue notices when employees are required to submit a current driver's license on a weekly basis.<br><br>**NOTE:** The findings described in the column to the left refer to food handler's cards, not missing driver's licenses. See the corrective action plan above regarding the corrective action plan for food handlers permits. |

**Introduction**

We require that you submit a plan of correction for each deficiency listed on the inspection report form. Your plan of correction must be submitted to the DOH within ten business days of receipt of the list of deficiencies.

You are required to respond to the Inspection Report with Noted Deficiencies by submitting a plan of correction (POC). Be sure to refer to the deficiency number. If you include exhibits, identify them and refer to them as such in your POC.

**Descriptive Content**
Your plan of correction must provide a step-by-step description of the methods to correct each deficient practice to prevent recurrence and provide information that ensures the intent of the regulation is met.

An acceptable plan of correction must contain the following elements:

- The plan of correcting the specific deficiency.
- The procedure for implementing the acceptable plan of correction for the specific deficiency cited.
- The monitoring procedure to ensure that the plan of correction is effective, and that specific deficiency cited remains corrected and/or in compliance with the regulatory requirements.
- The title of the person responsible for implementing the acceptable plan of correction.

Simply stating that a deficiency has been "corrected" is not acceptable. If a deficiency has already been corrected, the plan of correction must include the following:

- How the deficiency was corrected,
- The completion date (date the correction was accomplished),
- How the plan of correction will prevent possible recurrence of the deficiency.

**Completion Dates**
The POC must include a completion date that is realistic and coinciding with the amount of time your facility will need to correct the deficiency. Direct care issues must be corrected immediately and monitored appropriately. Some deficiencies may require a staged plan to accomplish total correction. Deficiencies that require bids, remodeling, replacement of equipment, etc., may need more time to accomplish correction; the target completion date, however, should be within a reasonable and mutually agreeable timeframe.

**Continued Monitoring**
Each plan of correction must indicate the appropriate person, either by position or title, who will be responsible for monitoring the correction of the deficiency to prevent recurrence.

**Checklist:**

- Before submitting your plan of correction, please use the checklist below to prevent delays.
- Have you provided a plan of correction for each deficiency listed?
- Does each plan of correction show a completion date of when the deficiency will be corrected?
- Is each plan descriptive as to how the correction will be accomplished?

- Have you indicated what staff position will monitor the correction of each deficiency?
- If you included any attachments, have they been identified with the corresponding deficiency number or identified with the page number to which they are associated?

Your plan of correction will be returned to you for proper completion if not filled out according to these guidelines.

Note: Failure to submit an acceptable plan of correction may result in enforcement action.

**Approval of POC**
Your submitted POC will be reviewed for adequacy by DOH. If your POC does not adequately address the deficiencies in your inspection report, you will be sent a letter detailing why your POC was not accepted.

**Questions?**
Please review the cited regulation first. If you need clarification or have questions about deficiencies you must contact the inspector who conducted the onsite inspection, or you may contact the supervisor.

STATE OF WASHINGTON

# DEPARTMENT OF HEALTH

2/18/2025

Edie Dibble
Two Rivers Landing
PO Box 959
Yakima, WA 98907-0959

Re: Exam Number: X2024-799
License Number: RTF.FS.60068796
**Approved Plan of Correction**
Date(s) of Inspection: 10/2/24-11/26/24

Dear Administrator:

This letter is to inform you that after careful review of the Plan of Correction (POC) you submitted for the exam recently conducted at your facility, the Department has determined that the POC is approved.

Based on the scope and severity of the deficiencies listed in your statement of deficiency report, the Department **will not** conduct an announced follow-up compliance visit to verify that all deficiencies have been corrected.

Reviewer:
Department of Health
HSQA/Office of Health Systems Oversight
PO Box 47874
Olympia, Washington 98504-7874